

LECTURE CONSOLIDATION SHEET

15 Collaterals · 12 Divergent Channels · 12 Muscle Regions · 12 Cutaneous Regions

Built from the completed Lecture 8 transcript + slide deck (not a pre-lecture draft)

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SCOPE FOR THIS WEEK

This week's content is structural/conceptual (no new organ acupoints) -- sections below follow the usual Interteaching format but are filled in from the completed lecture rather than pre-lecture predictions.

No CAM or MOA reading pages are assigned specifically for Week 8 in either the written syllabus excerpts or any "For Next Week" slide located in project files (checked both Lecture 7's forward-look and Lecture 8's own final slide, both editions of the deck). The AC300_CunAndChannels.pdf (CAM) and AC300_MOA_Channels.pdf (MOA) figure sets in project files are organized around the 12 PRIMARY organ meridians only (already used in Weeks 2-6) and contain no chapter/figures on Collaterals, Divergent Channels, Muscle Regions, or Cutaneous Regions -- consistent with the same gap already documented for Week 7's Extraordinary Vessels. This week's figures are therefore sourced entirely from Dr. Zhang's own lecture slides, as in Week 7.

A

Learning Targets

8 targets, pre/post confidence 1-5

B

Activate -- Week 7 Bridge

Confluent-points Q&A recap

C

Vocabulary Pre-Load

10 key terms

D

Anticipatory Questions

10 questions, 3 groups, starred = high-yield

IQ

Inter-Quiz

12 items, 3 checkpoints

E

Clinical Case

Reasoning + protocol

F

After-Lecture Synthesis

3-column synthesis

A Learning Targets

*Purpose: these define what you should be able to DO after review, not just recognize.
Since the lecture already happened, rate your Pre score honestly based on where you were BEFORE building these materials.*

I can state why there are 15 Collaterals rather than 12, using the front/back/side logic.

Pre: 1 2 3 4 5 Post: 1 2 3 4 5

I can name all 12 paired-meridian Luo-Connecting points and their locations (wrist vs. ankle rule).

Pre: 1 2 3 4 5 Post: 1 2 3 4 5

I can name the 3 “extra” collaterals (CV, GV, Spleen Great Luo) and explain what each covers.

Pre: 1 2 3 4 5 Post: 1 2 3 4 5

I can describe the Li-He-Chu-merge framework and apply it to any of the 12 Divergent Channels.

Pre: 1 2 3 4 5 Post: 1 2 3 4 5

I can explain why Divergent Channels have no points and no pertaining organ, unlike primary meridians.

Pre: 1 2 3 4 5 Post: 1 2 3 4 5

I can state the 4 structural pattern rules for the 12 Muscle Regions (Yang/Yin, Hand/Foot).

Pre: 1 2 3 4 5 Post: 1 2 3 4 5

I can describe the function of the Cutaneous Regions and the Su Wen disease-transmission order.

Pre: 1 2 3 4 5 Post: 1 2 3 4 5

I can identify at least 2 acupoints that carry both a Luo-Connecting AND a Confluent-point identity.

Pre: 1 2 3 4 5 Post: 1 2 3 4 5

Purpose: Dr. Zhang opened Lecture 8 with a live Q&A review of the 8 Extraordinary Vessels' confluent points before introducing new content. Test yourself on the same Q&A she used.

Q: Which vessel is called the “Belt” because it runs around the waist?

Q: True or false: Dai Mai is the only vessel that runs horizontally.

Q: Which two vessels have a confluent point that is the SAME as their starting point?

Q: Which three vessels share the lower abdomen as a common origin (“yi yuan san qi”)?

Q: True or false: the Du Meridian connects with the Bladder (zang-fu connection).

Q: Which channel is called the “Sea of All Yin Meridians”?

Q: Which channel is called the “Sea of All Yang Meridians”?

C Vocabulary Pre-Load

Purpose: load the Chinese/English terms this week's four systems are named with, before drilling content.

PINYIN	ENGLISH	NOTE
Luo Mai	Collateral / connecting vessel	The 15 Luo-Connecting points as a system
Jing Bie	Divergent Meridian	Runs deep, no points, no organ
Jing Jin	Muscle / Sinew Region	Wide band, muscles/joints only
Pi Bu	Cutaneous Region	Outermost layer, skin surface
Li	Beginning (of a divergent channel)	First of the 4-part framework
He	Organs/systems involved	2nd of the 4-part framework
Chu	Exiting (to the surface)	3rd of the 4-part framework
Dabao	SP 21, the Great Luo of the Spleen	Covers the lateral chest
Bi Syndrome	Painful obstruction syndrome	Key Muscle Region pathology
Yi Yuan San Qi	"One source, three branches"	Du/Ren/Chong (Week 7 recap term)

Purpose: these probe the reasoning behind this week's structure, not just recall.

STRUCTURAL

* Why does the Spleen (not another organ) get the Great/Major Collateral that covers the lateral chest?

How many total collaterals are there, and how is that number built from 12 + 2 + 1?

* What is the ONE thing all 12 Divergent Channels lack that every primary meridian has?

What does the “Chu” step of the Li-He-Chu-merge framework refer to?

CLINICAL

* Why can Stomach-channel points sometimes address heart-adjacent chest symptoms?

What clinical pattern do Muscle Regions treat that neither the primary meridian nor its Luo point directly addresses?

* Why might LU7 (Lieque) be chosen over another Lung-meridian point for a combined cough + irregular-menstruation presentation?

COMPARISON

How do Collaterals and Divergent Channels differ in depth, points, and function?

* How do the 4 Yang Muscle Region pattern-rule groups differ from the Yin groups (what do they each connect to)?

Where do the 3 Yin Cutaneous Regions sit relative to the 3 Yang Cutaneous Regions (anterior vs. posterior)?

Purpose: spaced retrieval, checked every 4 items instead of at the end.

Mastery: $\geq 10/12$ across 3 separate sessions on different days.

Date: _____ Start: _____ End: _____ Session score: ____/12

CHECKPOINT 1 -- Items 1-4: 15 Collaterals

1. [ACQ] Name the Luo point of the Stomach meridian and what it connects to.
2. [ACQ] Name the Luo point of the Kidney meridian and what it connects to.
3. [MAINT] What is the confluent point of Chong Mai, and is it also a Luo point?
4. [ACQ] Which collateral covers the lateral side of the chest, and from what point?

Self-check score this block: ____/4

CHECKPOINT 2 -- Items 5-8: Divergent Channels

5. [ACQ] What does the Kidney Divergent Channel cross that no other divergent channel does?
6. [ACQ] Which Divergent Channel passes through the heart on its way to the mouth and eye?
7. [MAINT] What is the pertaining organ of the Pericardium, and its connecting organ?
8. [ACQ] What do all 6 Yin Divergent Channels eventually do?

Self-check score this block: ____/4

CHECKPOINT 3 -- Items 9-12: Muscle & Cutaneous Regions

9. [ACQ] What do all 3 Yang Muscle Regions of the Foot connect with?
10. [ACQ] What do all 3 Yin Muscle Regions of the Hand connect with?
11. [ACQ] State the Su Wen disease-transmission order from skin to organ.
12. [MAINT] Which vessel is the Sea of All Yang Meridians (Week 7 recap)?

Self-check score this block: ____/4

1. ST 40 Fenglong -- connects to the Spleen meridian.
2. KI 4 Dazhong -- connects to the Bladder meridian.
3. SP 4 Gongsun is the confluent point of Chong Mai, and yes -- it is also the Luo-Connecting point of the Spleen meridian.
4. The Great (Major) Collateral of the Spleen, from Dabao (SP 21).
5. It crosses the Dai (Belt) Meridian, at the level of T7.
6. The Stomach Divergent Channel.
7. Pertaining organ: Pericardium. Connecting organ: Sanjiao (Triple Energizer) -- an interior-exterior pair.
8. They merge into their paired Yang primary meridian -- none resurfaces as a separate Yin pathway.
9. The eyes.
10. The thoracic cavity (chest).
11. Skin -> Collaterals -> Meridians -> Fu organs -> Zang organs.
12. Du Mai (Governor Vessel).

Session 1: ____/12 Session 2: ____/12 Session 3: ____/12 Criterion met ($\geq 10/12 \times 3$): []

E Clinical Case -- Apply It

Purpose: bridges structural theory to clinical reasoning.

Case Vignette: Ms. Alvarez, 38 years old

Ms. Alvarez, 38, presents with several weeks of intermittent right-sided lateral hip and outer-knee pain that worsens with prolonged standing, plus mild stiffness first thing in the morning. She denies any digestive, urinary, or gynecological complaints, and her only other note is that she “never really feels it in one exact spot” -- the discomfort seems to move along a band down the side of her leg rather than sitting at a single point.

Given that her pain follows a BAND rather than a single point, and there's no organ-level complaint, which of this week's 4 systems (Collaterals, Divergent Channels, Muscle Regions, Cutaneous Regions) best fits this presentation, and why?

Now that you've studied the Muscle Regions in depth: which specific Muscle Region (by meridian) best matches a lateral hip-to-knee band, and what binding points would you expect to be tender?

F. After-Lecture Synthesis

Before I thought...	Now I know...	Still confused about...

Update Section A confidence scores now, then file this sheet with your Week 8 Study Guide.